

SAME SIZE ARTWORK
LEAFLET SIZE: 140 mm x 88 mm

Route of administration : Topical		For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory	
SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE (S) Acute overdosage is very unlikely to occur, however, in the case of chronic overdosage or misuse, the features of hypercortisolism may appear and in this situation topical steroids should be discontinued gradually. However, because of the risk of acute adrenal suppression this should be done under medical supervision.		GLEVATE™ CREAM	
		SPACE FOR PHARMACODE	
PACKING / PACK SIZES: 15 g tube		COMPOSITION Clobetasol Propionate BP 0.05 % Cream base q.s.	
SHELF LIFE: 24 months		THERAPEUTIC CLASS T0200	
STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS Store below 30° C. Protect from light.		PRODUCT DESCRIPTION A Printed carton containing a leaflet and a printed aluminium collapsible tube containing white semi-solid cream.	
MAL06051319 AZ		PHARMACOLOGY Mechanism of action Clobetasol Propionate has both anti-inflammatory and immunosuppressive activity. Thus Clobetasol inhibits the adherence of neutrophils and onocyte macrophages to the capillary endothelial cells of the inflamed area. The drug blocks the effect of macrophage migration for inhibitory factor and decreases the activation of plasminogen to plasmin. Finally, by inhibition of phospholipase A2 activity, via formation of lipocortin, Clobetasol reduces the formation of prostaglandins and leukotrienes in the local tissue.	
BRU No.: BRU26023662P			
DATE OF REVISION: 01 Apr 2026			
Manufactured by: glenmark PHARMACEUTICALS LTD. Plot No. E-37, 39, D-Road, MDC, Satpur, Nashik - 422 007, Maharashtra State, India. TM Trade Mark		Product Registration Holder: Glenmark Pharmaceuticals (Malaysia) Sdn Bhd (660397-W) D-31-02, Menara Suezcap 1, No. 2, Jalan Kerinchi, 59200 Kuala Lumpur, Malaysia	
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Pharmacokinetics Clobetasol Propionate is absorbed through the skin in varying proportions of the applied dose depending upon the severity of the damage to the stratum corneum barrier that has resulted from the skin disease being treated. It is probable that no more than 5% of the applied dose is absorbed from normal skin but much may be absorbed from inflamed skin or under occlusion which greatly increases the permeability of the stratum corneum.	when treating such conditions as psoriasis, discoid lupus erythematosus and severe eczema. If applied to the eyelids, care is needed to ensure that the preparation does not enter the eye, as glaucoma might result. If Glevate Cream does enter the eye, the affected eye should be bathed in copious amounts of water.
INDICATIONS Glevate Cream is of particular value when used in short courses for the treatment of more resistant dermatoses such as psoriasis, (excluding widespread plaque psoriasis) recalcitrant eczema, lichen planus, discoid lupus erythematosus and other skin conditions which do not respond satisfactorily to less active corticosteroids.	Topical steroids may be hazardous in psoriasis for a number of reasons including rebound relapses, development of tolerance, risk of generalised pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis careful patient supervision is important.
CONTRAINDICATIONS It is contraindicated in patients who are hypersensitive to any of the ingredients contained in it.	Appropriate antimicrobial therapy should be used whenever treating inflammatory lesions which have become infected. Any spread of infection requires withdrawal of topical corticosteroid therapy and systemic administration of antimicrobial agents.
As with other topical corticosteroids, this product should not be used in rosacea, acne, perioral dermatitis, primary cutaneous viral infections (herpes simplex, chickenpox), fungal infections (candidiasis, tinea) and bacterial infections.	Bacterial infection is encouraged by the warm, moist conditions induced by occlusive dressings, and so the skin should be cleansed before a fresh dressing is applied.
SIDE EFFECTS / ADVERSE REACTIONS Prolonged and intensive treatment with highly active corticosteroid preparations may cause local atrophic changes, such as thinning, striae, and dilatation of the superficial blood vessels, particularly when occlusive dressings are used or when skin folds are involved.	DRUG INTERACTION None
There are reports of pigmentation changes and hypertrichosis with topical steroids. In rare instances, treatment of psoriasis with corticosteroids (or its withdrawal) is thought to have provoked the pustular form of the disease. Glevate Cream is usually well tolerated, but if signs of hypersensitivity appear, application should be stopped immediately. Exacerbation of symptoms may occur.	USE IN PREGNANCY AND LACTATION Topical steroids should not be used extensively in pregnancy i.e. in large amounts or for prolonged periods. Because many drugs are excreted in human milk, caution should be exercised when administered to a nursing woman.
PRECAUTIONS / WARNINGS Long-term continuous therapy should be avoided where possible, particularly in infants and children, as adrenal suppression can occur even without occlusion. If Glevate Cream is required for use in children, it is recommended that the treatment should be reviewed weekly. It should be noted that the infant's napkin may act as an occlusive dressing. If used in childhood or on the face, courses should be limited if possible to five days and occlusion should not be used. The face, more than other areas of the body, may exhibit atrophic changes after prolonged treatment with potent topical corticosteroids. This must be borne in mind	RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION Apply sparingly to the affected area once or twice daily until improvement occurs. As with other highly active topical steroid formulations, therapy should be stopped when control is achieved. In the more responsive conditions, this may be within a few days. If a longer course is necessary, it is recommended that treatment should not be continued for more than four weeks without the patient's condition being reviewed. Repeated short courses of Glevate Cream may be used to control exacerbations. If continuous steroid treatment is necessary a less potent preparation should be used. In resistant lesions, especially with hyperkeratosis, the anti-inflammatory effects of these preparations can be enhanced, if required, by occluding the treatment area with polythene film. Overnight occlusions is generally adequate for satisfactory response. Thereafter, improvement can usually be maintained by application without occlusion.
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ICONGRAPHICS CODE:

PANTONE SHADE	
	PANTONE BLACK PROCESS C
Supersedes Artwork Code PE55820	

PHARMACODE: